



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : سماح عبدالحكيم احمد السيد

العنوان : ش الفوال صفط اللين
امبابه - الجيزة

الحي / المدينة : سته اكتوبر

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 06-10-2026

رقم البطاقة : 28111142102589 رقم التليفون : 01128139792

العمر (تاريخ الميلاد) : 14-11-1981 المهنة : حاصل علي دبلوم

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.
تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سماح عبدالحكيم احمد السيد National ID: 28111142102589	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


 جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 سماح
 عبد الحكيم احمد السيد
 ١١ من الفوال - صلفط اللين
 بولاق الدكرور - الجيزة
 ١٩٨١/١١/١٩ ٢٨١ ١١ ١٤ ٢١ ٢٥ ٨٩
 IR8419953

A13


 ٢٠٢٠/١٢ ٢٨١١١٤٢١٠٢٥٨٩
 حاصله على دبلوم معهد فنى
 علاقات صناعية
 أنثى متزوجة
 وليد محمود أحمد الوكيل
 البطاقة سارية حتى ٢٠٢٧/١٢/١٩


Name : Samah Abdulhakim Ahmed El-Sayed
Branch : Dokki
Ref. Physician Prof : Goustav Roussey Egypt

Date : 04-04-2026 10:35
Medical No. : 341926

Tc-99m MDP SKELETAL SCINTIGRAPHY

CLINICAL HISTORY

- 45 -year-old female patient with history of treated right breast cancer complains of bony pain ; referred for metastatic survey.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- Compared to the bone scan dated 1/7/2025, the current scan revealed still noted osseous lesion located at DV4 and symphysis pubis as well as right sterno-clavicular region and anterior end of bilateral lower ribs.
- New osseous lesion is noticed at trochanteric region of left femur, for CT correlation.
- Osteoarthritic changes around big joints.

IMPRESSION:

- Compared to the bone scan dated 1/7/2025, the current scan revealed still noted osseous lesions at DV4 and symphysis pubis as well as right sterno-clavicular articulation and anterior end of bilateral lower ribs, reflecting rather stationary course.
- New lesion is currently seen at the trochanteric region of left femur, for CT correlation.
- Follow-up recommended.

Much Obliged
Dr. Alaa Nagy ,MD,
Nuclear Medicine

Name: سماح عبد الحكيم احمد السيد		
MR. : 1792		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient has metastatic disease involving the vertebrae, pubic bone, and the right fourth rib. The patient is a 44-year-old individual with right-sided invasive ductal carcinoma (IDC) of the breast, estrogen-receptor (ER) 7/8 positive, progesterone-receptor (PR) 7/8 positive, HER2-negative, and Ki-67 71 %. The diagnosis was made in 2018. Neoadjuvant chemotherapy consisting of four cycles of anthracycline (AC) and four cycles of taxane (TXT) was administered, followed by right-sided modified radical mastectomy (MRM) in June 2018. Adjuvant tamoxifen 20 mg was started. A bone scan performed in June 2023 demonstrated bone metastases; palliative radiotherapy was delivered to the spine (four fractions) in 2023 and to the pelvis (three fractions) in 2024. Systemic therapy was initiated on 24 June 2023 with palbociclib (IBRANCE) 125 mg, zoledronic acid (ZOLADEX) 10.8 mg, letrozole (FEMARA), and zoledronic acid (ZOMETA) 4 mg, with subsequent bone scan for response assessment. The patient presented with headache and was scheduled for brain MRI; computed tomography (CT) and bone scan (BS) were pending while awaiting continuation of letrozole and palbociclib 125 mg. A CT of the chest and bone scan were ordered without prior comparison; approval for continuation of palbociclib with letrozole was not obtained on that day. The patient received zoledronic acid (ZOLADEX) and zoledronic acid (ZOMETA) after laboratory results. The patient complained of left hip pain; on examination the patient was limping and had tenderness over the left hip. Imaging revealed a mild progressive right axillary mass and a newly developed left femoral trochanteric lesion on bone scan. The plan included palliative radiotherapy to the left hip and multidisciplinary discussion to consider switching to aromasin (exemestane) and faslodex (fulvestrant). The patient was started on Afinitor (everolimus) with a cycle 2 designation, together with aromasin after laboratory results, and received zoledronic acid (ZOLADEX) on the same day. Zoledronic acid (ZOMETA) was scheduled for dispensing on 1 September 2026. The patient has completed two cycles of Afinitor plus aromasin to date.		
تحريرا في ,		
	التاريخ :	19-09-2026
	الوقت :	
	الطبيب المعالج :	سيلفيا سامي باسيلى مرجان
يعتمد ,		

Patient's Name:

samah abd elhakim ahmed



Date: 2026-04-14

Referred by: Prof.Dr. NCAP+C

Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported breast carcinoma .

AS COMPARED TO PREVIOUS STUDY DATED 4/11/2025, THE CURRENT STUDY REVELED

(I) Chest:

- ☐ Right mastectomy with clear operative bed .
- ☐ Mild progressive course of the previously mentioned right axillary tail partially calcified mass lesion measures 3.3 x 2 cm at current study compared to 24x17mm .
- ☐ Few subcentemtric mediastinal lymph nodes , the largest seen at prevascular group measures 7 x 4 mm .
- ☐ No sizable pulmonary nodules or masses .
- ☐ Clear both lung fields .no consolidation patches
- ☐ No pleural or pericardial collections seen.
- ☐ No gross cardiac abnormality.

(II) Abdomen and pelvis:

- ☐ The liver is enlarged in size showing homogenous CT density , no focal lesions
- ☐ Patent and homogenously enhancing portal vein.
- ☐ No intra or extra hepatic biliary radicles dilatation.
- ☐ Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- ☐ Normal CT appearance of the spleen .
- ☐ Regular filling of the urinary bladder showing no stones, masses .
- ☐ Unremarkable other pelvic organs.
- ☐ No retro-crural, para-aortic or pelvic lymphadenopathy.
- ☐ No ascites.
- ☐ Scattered sclerotic bony lesions of spine , pubic and iliac bones ..
- ☐ No vertebral structural collapse .

OPINION:.

- Mild progressive course of the right axillary tail mass for sonomammographic correlation
- Stationary rest of study ..

Dr.Reham Khedr, MD

Patient's Name: samah abd elhakim ahmed
Date: 2026-04-14
Referred by: Prof.Dr. NCAP+C
Hospital: Goustave Roussey International Egypt

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FINDINGS:

Clinically reported breast carcinoma .

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- Right axillary tail partially calcified mass measures 3.3 x 2 cm .
- Few subcentemtric mediastinal lymph nodes , the largest seen at prevascular group measures 7 x 4 mm .
- No sizable pulmonary nodules or masses .
- Clear both lung fields .no consolidation patches
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

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- The liver is enlarged in size showing homogenous CT density , no focal lesions
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.



الهيئة العامة للغذاء والدواء
Saudi Food & Drug Authority

الهيئة العامة للغذاء والدواء
Saudi Food & Drug Authority

MDT FORM 24/6/2023 GA

Name	26 سالمه	MRN	576-5-25
Age	40	ID	5111881-5049
Menopausal status	Pre-menopausal	Phone	1151-89495
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

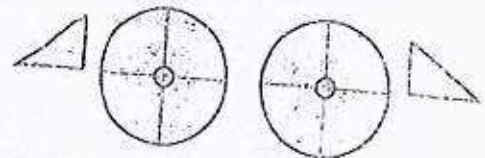
BX: 7/2018 CBS AX T2 Np/14

ER:	PR:	HER 2:	Ki67:
5/2018 7/18	7/18	none	< 10%

Sonogramography:

Views Workup: Bone scan - This bone mets
CT scan requested.

R



TNM

Recommendations: Repeat Newby 4 AC + Taxanes
6/2018 (R) MRM - then PORTH + Adj Tamoxifen (sys)
6/2023 Bone scan Bone mets. (Repeat positive RTH)
For AxicMKu/6 / AI / Zoladex / Zometa

MDT OUTCOME & PLAN

24/6/2023 ->
- CD Ku/6 - / Aromatase Inhibitor / Zoladex
- Zometa (mn (after dental exam)
- ~~Don't~~ bring report of CT

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name : Samah Abdulhakim Ahmed El-Sayed

Branch : Maadi

Ref.Physician Prof : Gostaf Roseh Hospital

Date : 01-07-2025 12:30

Medical No. : 140570

Ins.Company : التأمين الصحي

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- A 43-year-old lady, with history of right breast cancer, referred for metastatic work up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

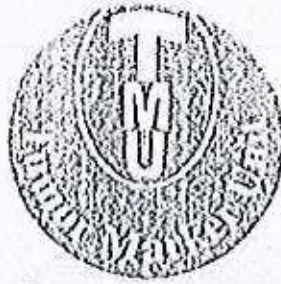
Considering last bone scan report dating 23/05/2023:

- The previously reported osseous lesions at left 5th rib anteriorly as well as LV3-4 are not currently visualized.
- Still noted enhanced tracer uptake at DV4 as well as pubic symphysis.
- No newly developed osseous lesions could be seen
- Degenerative changes are seen at the spine and big joints, more evident at right sternoclavicular region.

Impression:

- Still noted osseous lesions at DV4 and pubic bone with resolution of the ones at left 5th rib and lumbar vertebrae.
- No newly developed osseous lesions could be seen

*Much obliged
Dr. Alaa Nagi, MD
Consultant of nuclear medicine*



IMBIOLOGY PATHOLOGY
قسم الباثولوجيا - المستشفى المركزي

Patient Name: سماح محمد الحكيم

Lab.No:2018-546

Date: 5/2018

Sex: Female

Markers: ER, PR, Her 2 neu and Ki67.

Procedure:

Sections were prepared from paraffin blocks treated for (ER-clone SP1) (PR clone IE2) (Her 2-clone 445) and (Ki67- clone 30-9) automatically stained in Bench mark GX (DAKO) using LS Vector (1:1 TRA VIEW DAB) Detection Kit. Hematoxylin was used as counter stain.

Results:

- Examination of sections treated for ER and PR revealed moderate nuclear staining of 10% of tumor cells by chromogen.
- Examination of sections treated for Her 2 neu revealed no membranous staining of tumor cells by chromogen.
- Examination of sections treated for Ki67 revealed positive nuclear staining in 10% of tumor cells by chromogen.

DIAGNOSIS:

- POSITIVE FOR ER AND PR SCORE 7/8
- NEGATIVE FOR HER 2 NEU
- Ki67: 10%

PROF. DR. SANHA GABAL

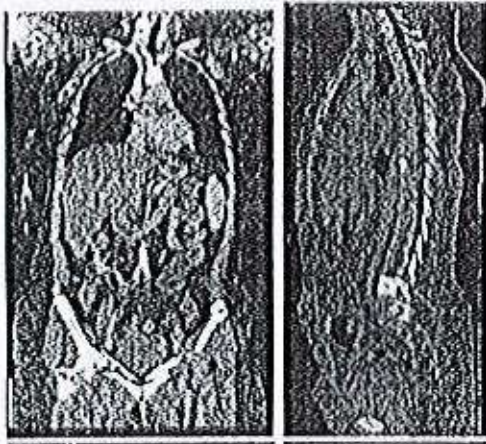
Name : Samah Abdel Hakeem

Date : 20/7/2025

MULTI-SLICE POST CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: - PRIMARY REPORT

Clinical History:

- Known case of breast cancer, compared to the previous study report dated 4-1-2025, the current study revealed:



Findings:

- The liver is still enlarged (19 cm) with homogenous low CT texture (fatty infiltration). No sizable focal lesions.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Mildly enlarged spleen (13.5 cm) with no focal lesions seen.
- Normal size and parenchymal thickness of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenal glands.
- Average filling of the urinary bladder with no definite masses or stones.
- No ascites.

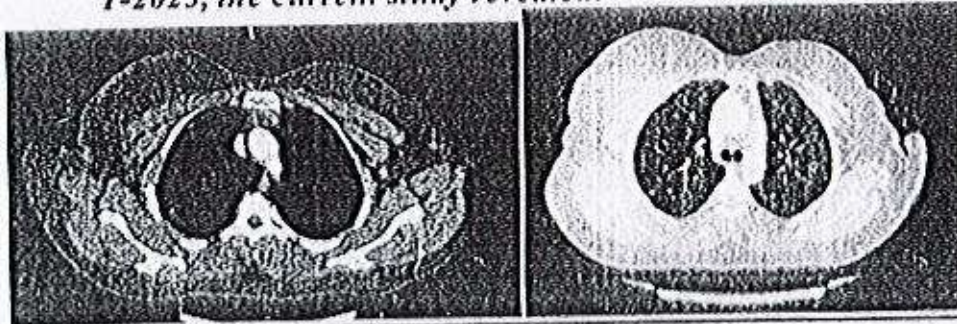
Name : Samah Abdel Hakeem

Date : 20-7-2025

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -
PRIMARY REPORT**

Clinical History:

- Known case of breast cancer, compared to the previous study report dated 4-1-2025, the current study revealed:



Findings:

- Clear operative bed with no sizable recurrent lesions or pathological axillary lymph nodes.
- Still noted right breast fat containing thick-walled lesion with marginal thick walls mostly representing traumatic fat necrosis. Another adjacent calcified lesion is also noted.
- Still noted left breast retro-areolar asymmetrical density.
- Still noted left upper lung lobe apico-posterior segment nodules, one is calcified.
- Resolution of the previously seen branching small nodules of ground glass attenuation associated adjacent tree in bud appearance seen implicating the right lower lobe.
- Resolution of the previously reported right upper lung lobe few patchy consolidative and ground glass opacities.

- Right upper lobe pulmonary nodule reaching 6.5 mm in diameter for correlation with unavailable previous films.
- No sizable hilar or mediastinal lymph nodes.
- Patent tracheo-bronchial tree.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.
- Normal CT appearance of the mediastinal vessels.

OPINION:

- *Clear operative bed with no sizable recurrent lesions or pathological axillary lymph nodes.*
- *Right upper lobe pulmonary nodule for follow up.*
- *Stationary course of the disease process.*

Marina Essam, MSc

- Still noted widespread axial and appendicular destructive mixed lytic and sclerotic osseous lesions.

Opinion:

- *Still noted widespread axial and appendicular destructive mixed lytic and sclerotic osseous lesions.*
- *Fatty hepatomegaly for clinical and lab correlation.*

Marina Essam, MSc

Patient's Name: samah abd elhakim ahmed
Date: 2026-04-14
Referred by: Prof.Dr. NCAP+C
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

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Normal CT appearance of the spleen .
Regular filling of the urinary bladder showing no stones, masses .
Unremarkable other pelvic organs.
No retro-crural, para-aortic or pelvic lymphadenopathy.
No ascites.
Scattered sclerotic bony lesions of spine , pubic and iliac bones ..
No vertebral structural collapse .

OPINION:

- Mild progressive course of the right axillary tail mass for sonomammographic correlation
- Stationary rest of study ..

Dr.Reham Khedr, MD

Name : Samah Abdulhakim Ahmed El-Sayed

Date : 04-04-2026 10:35

Branch : Dokki

Medical No. : 341926

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعافد مستشفيات

Tc-99m MDP SKELETAL SCINTIGRAPHY

- **CLINICAL HISTORY:** 43-year-old female patient; history of breast cancer, complains of bony pain; referred for metastatic survey.
- **Technique:** Anterior and posterior whole-body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- Few active osseous lesions are noticed at right sternoclavicular region, anterior end of bilateral lower ribs, DV4 and trochanteric region of right femur, likely metastatic.
- Osteoarthritic changes around big joints.

IMPRESSION:

- Few active osseous lesions as described above, likely metastatic, for radiological correlation.
- Follow-up is recommended.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

*Comparison
with
last study*
